BPC-157

Step-by-Step Reconstitution Process

- **Gather supplies:** BAC water, alcohol swabs, insulin syringe, vial
- **Removing plastic cap.** Carefully pop off the plastic cap, do not remove metal flange.
- **Clean both vials:** Wipe the rubber stoppers with alcohol swabs
- **Draw BAC water:** Pull your chosen amount into the syringe
- **Inject slowly:** Add the water to the peptide vial, aiming it at the glass wall—not directly onto the powder
- **Swirl gently:** Roll the vial between your palms. Never shake.
- **Wait:** Let it sit 5-10 minutes. Solution should be crystal clear.
- **Store:** Refrigerate immediately. Good for 28-30 days once reconstituted.

If it's cloudy or won't dissolve: Let it sit longer. Try gentle warming (under your arm or in lukewarm water). If still cloudy after 30 minutes, contact your source.

STANDARD PROTOCOL

Dose: 250 to 500 mcg per day
Frequency: Once or twice daily
Timing: Morning and/or evening (no fasting required)
Administration: Subcutaneous injection (preferred for localized injuries) or oral
Duration: 4 to 8 weeks for active healing

BY BODY WEIGHT

- Under 150 lbs: 200 to 300 mcg daily
- 150 to 200 lbs: 300 to 400 mcg daily
- Over 200 lbs: 400 to 500 mcg daily

BY APPLICATION

- Acute injury recovery: 400 to 500 mcg daily for 2 to 4 weeks, then reduce
- Chronic injury support: 250 to 300 mcg daily for 6 to 8 weeks
- Gut healing: 250 to 500 mcg daily, oral administration preferred
- Post-surgical recovery: 300 to 400 mcg daily (with surgeon approval)

RECONSTITUTION (standard)

Vial size	5 mg
BAC water	2 mL
Concentration	250 mcg per 10 units on insulin syringe

Where to Inject

BPC-157 amplifies existing repair signals at damaged tissue rather than creating new ones. With a half-life of less than 30 minutes, injection proximity matters.

For a specific musculoskeletal injury, inject near the injury site for maximum concentration at the target tissue before it clears. For multiple injuries or systemic inflammation, subcutaneous anywhere works because BPC-157 still circulates and reaches damaged tissue throughout the body. For gut-specific issues, oral administration delivers it directly to gastrointestinal tissue where it is uniquely stable.

Stacking Context

BPC-157 + TB-500 (Wolverine Stack)

The most common pairing. BPC-157 handles angiogenesis, meaning it builds new blood vessels and improves nutrient delivery to damaged tissue. TB-500 handles cell migration and repair work through upregulation of actin. Different mechanisms, complementary effects.

- BPC-157: 250 to 500 mcg daily (subQ)
- TB-500: 250 to 500 mcg daily (subQ) or 2 to 5 mg twice weekly
- Can be run together from day one.

Both BPC-157 and TB-500 promote angiogenesis. The same contraindications apply to both. If one is contraindicated due to cancer history, the other is too.

Common Questions

Do I need to inject near the injury site?

For a specific musculoskeletal injury, yes, injecting near the site gives you maximum concentration before the peptide clears. For multiple injuries or systemic inflammation, subcutaneous anywhere works because it still circulates and finds damaged tissue.

Does BPC-157 cause cancer?

No study has demonstrated that BPC-157 causes cancer. A 2025 review found that it actually inhibits uncontrolled cell proliferation in tumor contexts in animal models. However, it does upregulate VEGFR2, which is the same pathway tumors use to build blood supply. It is contraindicated with active malignancy. No cancer history and no active masses means low theoretical risk. Active tumors or recent cancer history means do not use until cleared by imaging and your oncologist.

Does oral BPC-157 work for gut health?

It is uniquely stable in gastric juice and animal studies show strong effects on structural gut damage like ulcers, NSAID-induced lesions, and intestinal permeability. Oral administration delivers it directly where it is needed. However, it does not address vague gut health concerns like bloating, microbiome imbalance, or food sensitivities. Fix the dietary foundation first.

Do I need to cycle BPC-157?

No. BPC-157 is a reactionary compound. Use it when tissue is damaged, stop when healing is achieved. Standard runs are 4 to 8 weeks for active healing, extendable to 8 to 12 weeks for deeper tissue repair.

Can I take it at any time of day? Do I need to fast?

No fasting requirement. No timing considerations. BPC-157 does not interact with food absorption the way growth hormone peptides do. Just be consistent.

Can I start BPC-157 and TB-500 together?

Yes. They can be run together from day one. There is no reason to separate them.